

APPELLANT'S OUTLINES OF EVIDENCE

Served on the Respondent under direction 2 of the Further Directions Order (3) dated 19 August 2026, one A4 page per witness.

Direction 2 material. Served on the Respondent and not filed in the Industrial Registry. The Appellant's list of names of all witnesses is a direction 1 document and is filed in the Registry as a separate one-page document; it is not part of this bundle.

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2	Outline of evidence – Mr Cory Lea Shepherd (Appellant)	Direction 2 – served, not filed
3	Outline of evidence – Mr Cory Harrison-Jones	Direction 2 – served, not filed
4	Outline of evidence – Ms Patricia Conaghan	Direction 2 – served, not filed

Each document is reproduced exactly as served. The outlines state the topics on which each witness will give oral evidence; they are not statements of evidence and are not verified. No expert report has been prepared for the purposes of this proceeding: the treating material relied upon is identified in the Appellant's schedule of medical documents, served with this bundle.

Dated 9 September 2026

Cory Lea Shepherd
Appellant, self-represented

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served 28 August 2026 (the Notice). The events at topics 2 to 7 are established by the facts admitted on 8 September 2026, except the emergency codes of 17 and 18 March 2024, which are not admitted and which I give from my own knowledge; my evidence on them is what they were from the console and their effect on me.

- 1. Employment and the role.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021 and full time from 16 October 2023, as a continuous shift worker. The Switchboard is the hospital's communications hub: in a Code Blue or MET call it is the Switchboard that activates and pages the response team, and a delay at the console is a delay in the emergency response. What each code is and what it requires of the operator, and what an urgent pathology result is, and what turns on the clinician being reached in time. From my own records, between 15 and 50 emergency codes go out in 24 hours and some 200 to 300 calls a shift, many from people in distress or crisis; urgent results relayed doctor to doctor; and the complaints that reach the console first, including those my manager did not action, which came back to me. The standing condition of the role over the period: from the moment I logged onto the board I was held responsible for what went wrong anywhere in that system, while the means of correcting it, the directory, the on-call roster, my own roster and my pay, were held by others. [Notice ¶¶ 1–16, 26–48, 182–223]
- 2. Sleep and fatigue.** My sleep on rotating shifts across 2023 and 2024 and the melatonin and temazepam prescribed on 16 November 2023; that after late shifts I would sleep in the car before driving home. The shifts of 17 and 18 March 2024: my travel time each way, finishing at 23:00 and starting at 06:00, and the rest and sleep actually available; that on the evening of 17 March I handled a MET call and, four minutes later, a Code Blue to the same bed, then a further MET call; that 18 March was a Monday day shift, the week's peak, and that between 06:00 and 14:00 I handled a Code Grey, two MET calls (one cancelled a minute later) and two neonatal MET calls, in that state, while answering that queue of urgent handover, distressed callers and complaints; the register of emergency codes kept at the Switchboard, its entries for 17 to 19 March 2024, and my screen capture of its March 2024 sheet; and that those shifts were not a staff-initiated swap, the only case in which, by Ms Forrest's email of 7 July 2026, the 8-hour agreement applied. The sequence that followed: my request of 8 April 2024; Ms Taylor's reply of 9 April that she had escalated it to Human Resources "to confirm policies around this"; no response until 1 May, when the fatigue payment was refused "after a consultation with payroll and Human Resources" on the basis of the 2020 agreement, so that the day of 19 March came from my own leave; the toolkit at clause 10.4.1 of the Operations Manual, sent to Ms Taylor, with no response; Ms Reese sending the fatigue guideline to Human Resources on 20 May 2024; that no fatigue risk assessment was made and no FRMS register kept; that Metro South has since stated in writing that those records "do not exist", that FRMS training applied only to clinical staff, and that fatigue management assessment at the Switchboard began only after 30 June 2024. [¶¶ 17–25, 211–227, 232–261, 263–271, 285]
- 3. Rostering: lates then earlies.** The pattern on my roster line of late shifts followed by early shifts, and of nights, days off, then nights again, raised on 16 April 2024; that rostering errors on my line recurred across roster periods; and Ms Reese's email to Human Resources of 10 May 2024 stating "a few rostering errors made by Chloe with regards to Cory's line in past rosters"; and that I was not told my roster proposal would not be considered, and learned of that email only from the Respondent's disclosure in July 2025. [¶¶ 211–223]
- 4. Pay and entitlements.** The public holidays I was not rostered between February and April 2024 and the difference to my pay. The loop: on 8 April 2024 I asked Ms Taylor to review my pay; on 9 April she directed me to raise the older dates through MyHR payroll enquiries; Payroll told me the payments had to be processed by my manager; on 3 May Payroll wrote to her, copying me, to "submit an AVAC to correct these shifts for each fortnight"; when I followed up with Payroll on 10 May, Payroll on 13 May told me to "speak to your Line Manager"; on 21 May she was still "waiting payroll confirmation"; on 28 May I was asked to sign a validation of claims older than three months. That under the payroll process only my manager could submit the AVAC. The February 2024 special pandemic leave application: what I attached, the two declines, and the approval on the same material; and the effect on me of being unable to have those errors resolved. [¶¶ 114–142, 182–210, 242–246]
- 5. The database, the misdirected calls, and what I could not fix.** That from 18 July 2023 operators could no longer correct database entries and the changes book was removed, so a wrong number waited on Ms Stibbard or Ms Taylor and after hours had to "wait until either Chloe or myself are back"; the directives of 15 April, 19 April and 9 May 2024, and that I was not consulted before any of them. That from 15 April 2024, when Ms Taylor notified the department that she and Ms Stibbard had placed themselves on after-hours call, departments and clinicians ringing during my shifts with pager and phone updates asked for her, and on many occasions neither they nor I could have the update or issue resolved during the shift. The MASPER Registrar's "Switchboard issues" emails to Ms Taylor of 3 May 2024 at 3:06 pm and 8 May at 5:28 pm, the nine occasions they log, including calls put through to the wrong team and the MET call team ringing because "switchboard could not tell them where VHUB was"; that her first response came on 9 May at 9:20 am, five days, eighteen hours and fourteen minutes after the first, asking Dr Wong for her business hours because they were "not provided on the rosters" and that her phone was "switched off"; and at 10:15 am telling the team of "many ongoing issues" about calls "transferred to the wrong medical teams"; the Integrated Respiratory Service writing on 15 May and again on 20 May 2024, when it said "we can not help patients or other clinical staff"; and that on each occasion the delay built up while the correction waited on someone else, and what that meant for the codes and calls I was handling at the time. [¶¶ 39–113]
- 6. Ms Taylor's hours: the question, the retraction request, and the answer.** That neither I nor the department knew her office hours; my email of 15 May 2024 at 1:15 pm asking her to share them; Ms Reese's reply that evening asking me to retract it, and what followed; that two days later, on 17 May 2024 at 9:30 am, Ms Taylor sent the whole department an email stating "Otherwise my hours are from 06:30-14:30", the very information I had asked for; and Ms Reese's reply of 21 May 2024 asking me to identify the directives I was concerned about. [¶¶ 70–88]
- 7. The complaints, and what followed them.** The communication book entry, a reminder about the monthly update of doctors' contact numbers, and its removal; my email of 16 June 2023 about consultation on changes; the concerns I raised on 7 August 2023 and the responses; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); my interest in becoming the Switchboard union delegate and what followed; and that, on Metro South's own statements, no safety assessment, investigation or change of procedure followed any complaint I made about misdirected calls, directory accuracy or fatigue before 30 June 2024, which is what Dr Hawes recorded on 1 July 2024 as "they don't listen to his complaints". What I said on these matters is in my emails; I never said anything hostile about Ms Taylor or any other member of staff. [¶¶ 143–153, 155–181, 266–267, 287–294]
- 8. My health before June 2024.** That before 18 June 2024 I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good. The ADHD and anxiety noted in my history in October 2022, and the psychiatrist referral to Dr Amini renewed on 16 May 2024, the day after the request that I retract my email, listing the medications I was then taking, none of them an antidepressant or an anxiolytic, and for which no appointment could be obtained. [General-practice records disclosed by the Respondent; ¶¶ 76–77]
- 9. Onset, presentation and diagnosis.** That I last worked a shift on 3 June 2024; that on days in May 2024 and in the first week of June 2024 I drove to work, could not bring myself to enter the building, and called in sick from the car park. My symptoms from about 18 June 2024 and their effect on sleep, concentration, self-care and relationships, and on my capacity for work, from being certified unfit from 1 July 2024 to returning on reduced hours and the restrictions recorded in the Checklist of 3 July 2026. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies". Dr Hawes's certificates of 1 July and 8 September 2024 and his psychiatrist referral. My first consultation with Dr Krishnaiah on 24 October 2024, on that referral, when I was told I had major depressive disorder with anxiety state, confirmed by the practice in writing at 11:45 am that day, my fluoxetine increased and quetiapine added at night; his written report of 13 February 2025; and my treatment since. The matters at paragraphs 2 to 7 above, and their effect on me as each occurred and as they accumulated, are the matters I reported to each doctor, as their records show; the events those records name are the facts admitted on 8 September 2026 (Tabs M1 to M4). I offer no clinical or diagnostic opinion.
- 10. After 18 June 2024, and documents.** Meetings scheduled while I was on certified leave; my employment was ended under abandonment provisions with a separation date of 20 September 2024, while I held current certificates, and my later reinstatement; the months without pay after the injury and the claim, with the pay errors never resolved, and the leave since. Documents: rosters and payslips; my emails and messages with my line manager and the Director; the employer's leave and payroll records; and Metro South Health's letter of 5 June 2026.

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.

OUTLINE OF EVIDENCE – MR CORY HARRISON-JONES

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

Mr Harrison-Jones was employed part time at Logan Hospital Switchboard and worked as an operator alongside the Appellant. He was a member of the Switchboard team from 2022, and was still a member on 18 July 2023, when he received the email at item 4 below. He was on shift at the time of the events he describes. He has since left the department and is no longer employed by Metro South Hospital and Health Service; he is now an officer of the Queensland Ambulance Service. He is expected to give evidence of the following.

1. That a function of the Switchboard was to hold and maintain accurate contact and paging numbers for medical staff, including on-call doctors, so that calls and emergency notifications could be directed to the correct person.
2. What he observed, over a period of approximately three months in that time, of the Appellant working to obtain and record updates to medical contact numbers; that the monthly task of updating the spreadsheet of number changes, allocated to a shift, was left undone over that period; that the Appellant placed a reminder in the Communication Book about the allocation of that monthly task; and that he was on shift and saw the Appellant check the contact numbers and write that reminder on handover.
3. That the Communication Book was kept in the operators' room and used by the Switchboard team to communicate and for matters of handover; that he read it in the course of handover during the period he worked at the Switchboard; and that he did not see in it any entry made by the Appellant that criticised any person, directly or indirectly.
4. That he was a recipient of the email sent by Ms Stibbard to the Switchboard team on 18 July 2023 stating that she would be removing everyone's access to the database and that changes were to be sent to her directly.
5. That after access was restricted operators could not correct an entry themselves and had to wait for Ms Stibbard or Ms Taylor to make the change.
6. An occasion on which he was present in the Switchboard room at the commencement of a shift, when the Appellant had arrived and was reading the handover and had not yet signed on: what Ms Taylor said to the Appellant about the removal of a reminder, the sequence in which each of them spoke, and the manner and volume in which it was said, from his own observation.
7. That he did not at any time hear the Appellant say anything hostile about Ms Taylor or about any other member of staff.

This outline is a summary of the evidence the witness is expected to give. It is not a statement of the witness and is not signed by the witness.

OUTLINE OF EVIDENCE – MS PATRICIA CONAGHAN

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

Ms Conaghan is an Administration Officer in Switchboard Services at Logan Hospital and has worked in that department alongside the Appellant. She is expected to give evidence of the following.

1. That she and the Appellant each sought an additional two shifts per fortnight, as recorded in his email of 7 August 2023 to Ms Taylor, Ms Reese, Ms Conaghan and Ms Smith.
2. The calls the Switchboard receives and what they are: that a substantial proportion come from people who are distressed, frightened, angry or abusive; that some callers make threats, including threats of harm to the operator, and demand the name of the operator they are speaking to; that calls of that kind are a regular and continuing feature of the work at the console and not occasional events; that the operator on duty is the first person they reach; and that she observed the Appellant receiving such calls.
3. What she observed of clinicians and departments remaining on the line to express their frustration to the operator on duty, and treating the operator as accountable for matters that were the responsibility of others.
4. Occasions she observed of complaints being directed at the Appellant at the console, including an orthopaedic doctor who complained about the Appellant following a change in the way that doctor was to be contacted, and a clinician who made repeated complaints through the Switchboard.
5. The process for relaying urgent pathology results before and after the changes made to the Switchboard process: what the process was, what it became, and what she was told at the console by those calling back about results that had not reached the treating team.
6. What she observed of the Appellant updating pager and contact numbers at the console, and of pager and number updates left unactioned over extended periods.
7. What she was told at the console between 2 and 8 May 2024 by callers reporting that calls had reached the wrong medical team, including calls concerning emergency responses.
8. That during that period operators had no means of correcting an incorrect entry in the directory themselves.
9. That the Communication Book was used by the Switchboard team to communicate and for matters of handover; that she has read it in the course of handover throughout the period she has worked with the Appellant; and that she has not seen in it any entry made by the Appellant that criticised any person, directly or indirectly.
10. That she was not consulted about the change to after-hours on-call arrangements notified by Ms Taylor to the Switchboard on 15 April 2024, and was not asked to vote on that change.
11. That she was not consulted about, and was not balloted in relation to, changes to the Switchboard roster at any time before 30 June 2024.
12. Any change she observed in the Appellant at work over the period, from her own observation. She offers no opinion as to diagnosis or cause.
13. That in the time they have worked together she has not heard the Appellant speak of Ms Taylor, or of any other member of staff, in hostile terms.

This outline is a summary of the evidence the witness is expected to give. It is not a statement of the witness and is not signed by the witness.